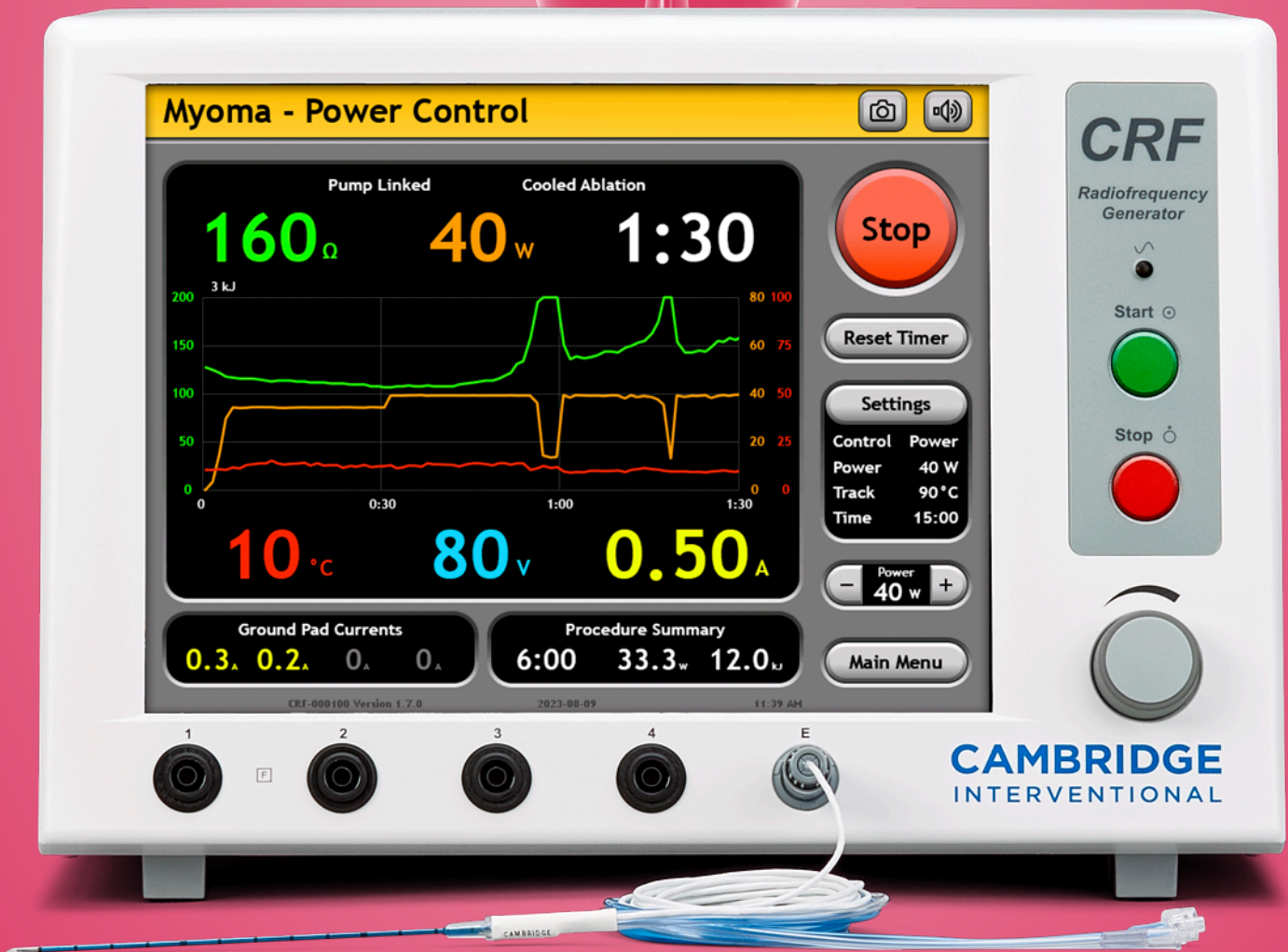


CAMBRIDGE
INTERVENTIONAL

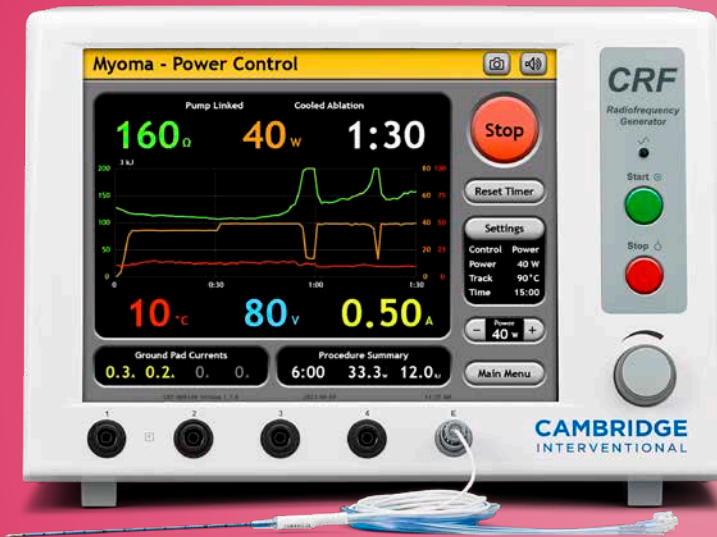
Myoma Radiofrequency Ablation



Minimally Invasive | Powerful | Easy to Use

CRF Ablation System

Real-time audiovisual feedback, automation, and electronic records help you focus on the patient and ultrasound imaging during leiomyoma ablation.



RF Generator

Hear and see impedance and temperature changes with up to 370 W of power, touch screen, energy meter, and data capture



Pump

Auto sync with generator for regular temperature checks



Footswitch

Fast, intuitive, hands-free de/activation

Cooled Electrodes

Internally-cooled, non-expanding, sharp electrodes provide superior control, visibility, robustness, and ergonomics to sculpt the ablation zone.¹

Lightweight, compact hub and cable reduce drag and disturbance

Sharp tip and smooth insulation penetrate tissue easily



Shaft marks for insertion depth

Steerable, robust, echogenic shaft

Procedure Steps

Radiofrequency ablation is a minimally invasive, outpatient alternative to hysterectomy or myomectomy for the treatment of uterine fibroids.^{1,3}

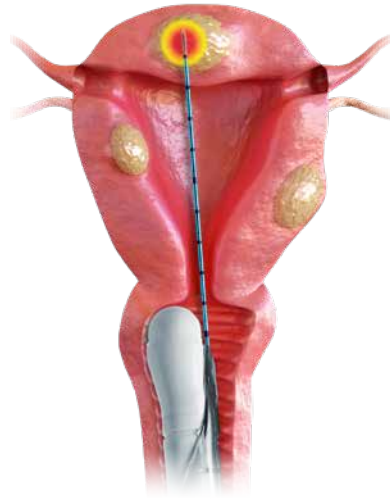
Target

Fibroids are located individually under intraoperative ultrasound image guidance.



Treat

A thin, sharp electrode delivers electrical energy to heat and break down fibroids from within.



Shrink

Successfully treated fibroids shrink gradually by 50-75% over 3-12 months, improving symptoms and quality of life.^{2,3,4}



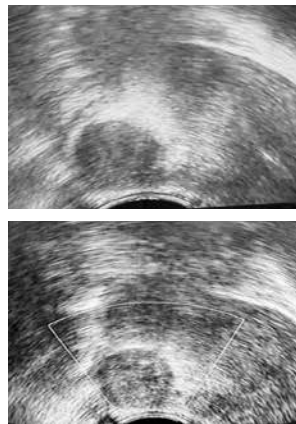
Case Report

Transvaginal RFA moving-shot myolysis using a Cambridge cooled electrode with 17G, 1 cm tip, 27 cm shaft, 40 W power. Courtesy of Dr. Domingo Molina.



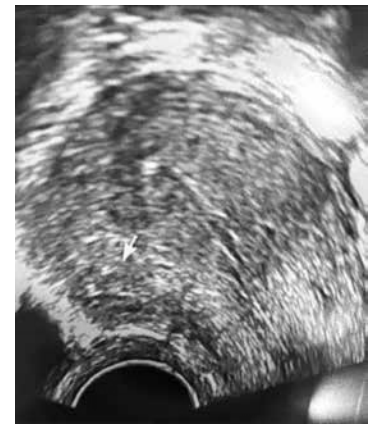
Echogenic Electrode

"Cambridge electrodes have excellent visibility throughout procedures."



Intraoperative

2.2 x 2.0 x 2.5 cm, 5.8 mL, type 5/6 myoma. No vascularity after RFA.



1 Year Follow Up

The myoma is almost undetectable one year after RFA.

Pioneering Expertise in RFA

Cambridge Interventional draws from 70 years of experience developing and manufacturing radiofrequency devices used to treat millions of patients.



1952
First RFA system
Radionics®
Bernard
Cosman^{6,7}

1980
First precision
temperature sensor
for RF probes
Prof. Eric Cosman⁶



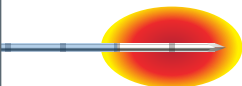
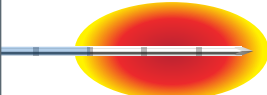
2005
Innovative
neurological
RFA devices
Cosman Medical⁷



1995
First cooled RFA
system for tumors
Radionics® Cool-tip®
Prof. Eric Cosman⁷



Present
Uterine
fibroid RFA
Cambridge
Interventional

17G Tip	Power	Time	Ablation Size ⁸	↕ x ↔	Shaft	Part Number
1 cm	40 W	15 sec		0.8 x 1.2 cm 0.3 mL	27 cm	CRF-U2510
2 cm	80 W	40 sec		1.5 x 2.6 cm 3.1 mL	27 cm	CRF-U2520
3 cm	130 W	45 sec		1.8 x 3.6 cm 6.1 mL	27 cm	CRF-U2530

Indications for Use: The CRF radiofrequency ablation system of Cambridge Interventional LLC ("Cambridge") is intended for use in percutaneous, laparoscopic and intraoperative coagulation and ablation of tissue.

Adverse Events: Reported adverse events or complications for RF ablation or coagulation procedures include, but are not limited to, the following: Abscess (pelvic), anesthesia reaction, arrhythmia, ascites, bile duct injury, bleeding (uterine, vaginal; spotting), blood clots (deep vein thrombosis), burn (uterine serosa), cardiac ischemia, death (miscarriage, intrauterine fetal death), device failure, device fracture in patient, diarrhea, edema, electric shock, electromagnetic interference with other devices and/or cardiac pacemakers, fever, fistula (rectouterine), general clinical residual risks for surgical procedures (flu-like symptoms, constipation, headache, inflammation, malaise), hematoma, hemorrhage, hypertension, hypotension, infection (urinary tract infection), nausea/vomiting, necessity for more invasive surgery (including open surgery, hysterectomy, demolitive surgery, removal of free intrauterine fibroid) if complications occur, nerve injury, organ damage (vagina, uterus, uterine wall, cervix, intestine, bladder; intrauterine adhesions; Asherman's syndrome; uterine rupture, window, atony, serosal laceration; placenta previa, accreta, abruption; invasive placentation; fetal growth restriction; unknown risk to future pregnancy), pain (abdominopelvic, cramping, dysmenorrhea), perforation (vagina, uterus, intestine, colon, bladder), peritonitis, skin burn, tumor recurrence (fibroid regrowth), tumor seeding, urinary retention/incontinence/leakage, vasovagal reaction (dizziness, fatigue), vessel injury, wound discharge (vaginal discharge, fibroid expulsion, sloughing). The long-term risks of RF ablations have not been established. RF ablation procedures are not recommended for pedunculated or giant fibroids, potentially malignant uterine tumors, or pregnant patients. Potential risks to the patient and/or fetus have not been established. If you become pregnant following uterine RFA, contact your doctor immediately. See device IFU for additional warnings and precautions.^{1,3,4,5}

Disclaimer: Rx only. Read the instructions for use ("IFU") of all medical devices prior to use. Clinical results, ablation sizes, costs, and financial/insurance coverage may vary and are not guaranteed. The information contained in the multimedia content that is contained or referenced herein or that is posted on the Cambridge Interventional website ("Content") is for general informational purposes only and is not intended to be a substitute for professional medical advice, diagnosis, or treatment; standards of medical care or training; or the instructions, indications, and contraindications for use of Cambridge Interventional devices or any other medical devices. All information is provided in good faith, however Cambridge makes no representation or warranty of any kind, express or implied regarding the accuracy, applicability, fitness, or completeness of this information; of opinions expressed; of third-party publications referenced or summarized; or of third-party services presented. Always seek the advice of your physician about a medical condition. Never disregard professional medical advice, or delay in seeking it, because of something you have read or seen in this Content.

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- Radionics® is a registered trademark of Integra. Cool-tip® is a registered trademark of Covidien AG. Cosman Medical LLC is owned by Boston Scientific. No affiliation with, or endorsement by, these companies is implied.
- Average size (std. dev. up to 8% of mean width and length, up to 16% of average volume; N=3-4) in 15-25°C ex vivo porcine muscle with 17-gauge cooled electrode position fixed. Clinical ablation size may differ, depending on tissue biology, vascular flow, proximity to vasculature and ducts, generator output level and time, and electrode temperature, size, spacing, and alignment.

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